جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : امل عصام عبدالحميد احمد
المحافظة : القاهره **الحي / المدينة :** اخري **العنوان :** الجيزه
رقم البطاقة : 27109040102667 **رقم التليفون :** 01156668921 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 04-09-1971 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امل عصام عبدالحميد احمد
National ID: 27109040102667

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

كود المستشفى: **102233**

وزارة الصحة و السكان
الامانة العامة للمراكز الطبية المتخصصة
جوستاف روسي انترناشيونال - مصر (EPE) *

فاتورة علاج رقم ٢٢٣٥٥٢٤٤

تقر مستشفى جوستاف روسي انترناشيونال - مصر (EPE) * بأن أصول مستندات الصرف ومؤيداتها القانونية صحيحة وعلى مسئوليتنا.
وتم حفظها بغرفة الحفظ لدينا وتحت تصرف الجهاز المركزي للمحاسبات واننا لم ولن نتقدم بتلك الفاتورة لأي جهة اخرى للصرف.

اسم المريض امل عصام عبدالحميد احمد الرقم القومي ٢٧١٠٩٠٤٠١٠٢٦٦٧
قرار وزاري رقم ٢٠٢٦٤٠٠٤٤٨٨١ بتاريخ ٢٠٢٦-٠٧-٠١ بمبلغ ٣٠٠٠ مدة القرار ٩٠ يوم
تاريخ الفاتورة ٢٠٢٦-٠٨-٠٦ التشخيص سرطان بالثدي (مبادرة)
الإجراء علاج تدعيمي وتلطيفي ودوائي وابحاث الخصم (%) ٠

مدين	بيان	كمية	وحدة	تاريخ الصرف	ملاحظات
١٢٠	رسم قلب	1		06/08/2026	
٤٧٠	موجات صوتية على القلب	1		01/10/2026	
١٥٠	صورة دم كاملة	1		06/08/2026	
٢٤٠	الايدز	1		06/08/2026	
٢٤٠	بي سي ار	1		06/08/2026	
١٥٠	سيوله PTT	1		06/08/2026	
١٥٠	سيوله PT	1		06/08/2026	
٧٠	كرياتنين	1		06/08/2026	
٧٠	ايه ا ل تي	1		06/08/2026	
٢٤٠	Anti RH Ab	1		06/08/2026	
٧٠	بولينا	1		06/08/2026	

٧٠	ايه اس تي	1		06/08/2026	
١٥٠	صورة دم كاملة	1		01/10/2026	
٧٠	بيليروبين مباشر	1		01/10/2026	
٧٠	بيليروبين كلي	1		01/10/2026	
٧٠	ايه اس تي	1		01/10/2026	
٧٠	ايه ا ل تي	1		01/10/2026	
٧٠	بولينا	1		01/10/2026	
٧٠	كرياتنين	1		01/10/2026	
٢٦١٠	الاجمالـــــــــي				

وتفضلوا بقبول فائق الاحترام،

الموظف المختص	المراجع	رئيس حسابات المرضى	مدير الشئون المالية
عبدالله محمود	ساره السيد	فاروق ناجي	

جوستاف روسي - مصر
G R I - E





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

03/10/2026

MDT FORM

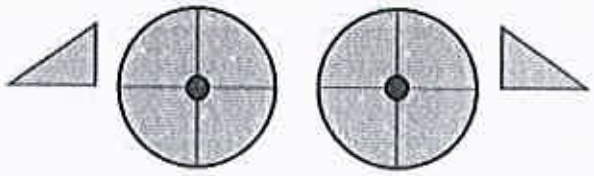
Name:	ال Essam Abd Elhamid احمد	**MRN:**	8389
Age:	55 Y	**ID:**	367/2026
Menopausal Sta	POST	**Phone:**	٢٧١٠٩٠٤٠١٠٢٦٦٧
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M:FREE S:APPENDECTOMY,2CS,D&C	**Reason Of MDT:**	NEW

Clinical Data:

BX: RT.IDC G2

ER: 8/8 **PR:** 0/8 **HER2:** AMPLIFIE **KI67:**

R L



TNM

Sonomammography:

Right breast :
* Scattered fibroglandular densities (ACR B).
* 9 o'clock segmetal grouped plemorphic microcalcifications, extended of area (10.5x3.3 cm). 4.5 cm away from nipple with underlying distortion
* This was confirmed by Complementary Ultrasound which showed 9 o'clock altered parenchyma consistent with microcalcifications noted in mammography.& spiculated hypochoic mass (1.4x1 cm), 7 cm away from the nipple, 0.3 cm from the skin, , another adjacent smaller mass closely related to the index mass, measures (0.4x0.2 cm)
Normal skin thickness and contour.
Normal nipple areola complexes.
* Indeterminate one axillary lymph node level 1, measures (2.4x0.8 cm) with increased cortical thickness 0.4 cm .

Mets Workup:

PET CT:FREE
BONE SCAN; FREE

History:

FNAC FROM LN:+VE FOR MALIGNANCY
CLINICALLY T2 N1 MO

Recommendations:

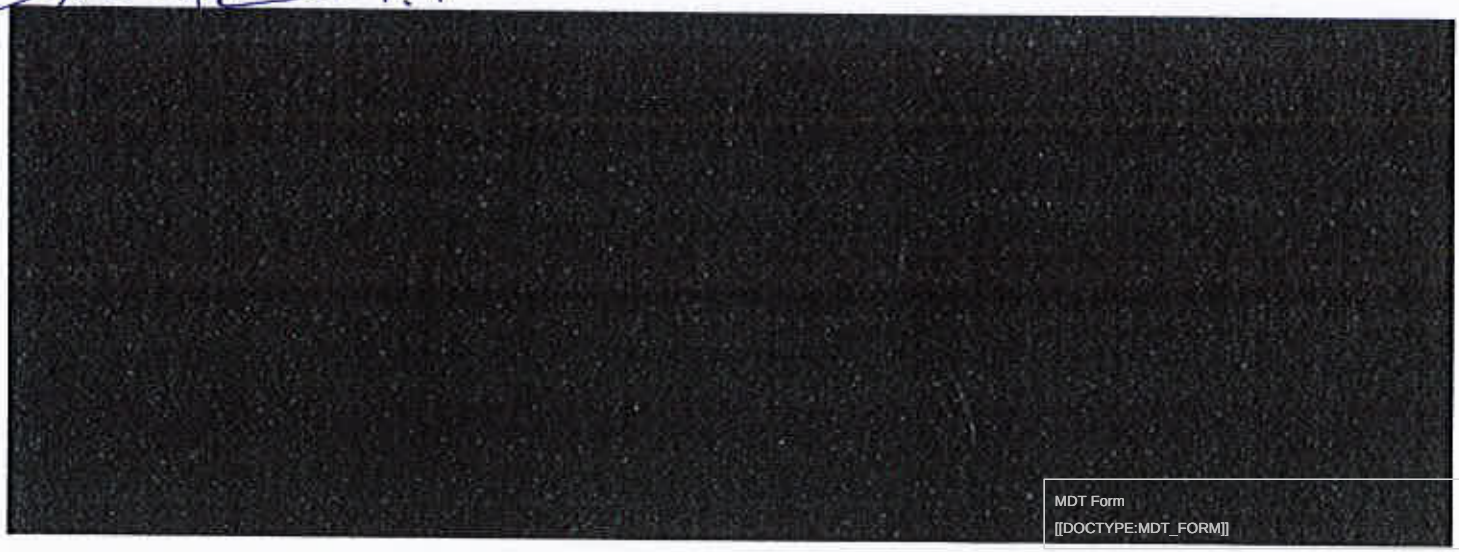
FOR POSSIBILTY OF BRACKETING WITH CLIPS THEN NACT FOLLOWED BY BCS -/+ LICAP & AXILLA ACCORDING TO RESPONSE

MDT Outcome & Plan

For BRACKETING WITH CLIPS + Porta Cath insertion
then Taxotere /Carboplatin + Phesgo
FOLLOWED BY RT BCS & AXILLA ACCORDING TO RESPONSE

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد البرقوقي				



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: امل عصام عبدالحميد احمد	
MR. : 8389	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 55-year-old individual who is postmenopausal. The clinical history notes that a stereotactic biopsy has not yet been performed; the patient will undergo wire localization of microcalcifications prior to surgery. The working diagnosis is right-sided invasive ductal carcinoma, clinical stage T2 N1, with a plan for neoadjuvant therapy consisting of six cycles of Taxotere, Carboplatin, and five phases of Phesgo.

Pathology from the forthcoming biopsy is reported as invasive ductal carcinoma, grade 2, with estrogen-receptor score 8/8, progesterone-receptor score 0/8, HER2 2+ with ISH amplification. Ki-67 is not specified. Imaging demonstrates a right-sided breast lesion and a PET-CT reveals a non-FDG-avid sclerotic lesion in the upper shaft of the right humerus, suggestive of a non-metabolic osseous focus.

The referring team recommends the possibility of bracketing wire localization of the area of microcalcifications, right breast-conserving surgery (BCS) with sentinel lymph-node biopsy (SLNB) and optional LICAP flap. The multidisciplinary team agreed to perform bracketing to include the area of microcalcifications and the inserted clip, followed by right wide-local excision (WLE) and SLNB.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

٨٢٨٩

Patient Name	: الأستاذه / امل عصام عبد الحميد	Request No.	: 13004805
Patient No.	: 27109040102667	Request Date	: 01/02/2026
Path No.	: SR-170/26	Verification Date	: 12/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Amended Report.

Clinical Data: Right breast 9 O'clock mass & right indeterminate axillary lymph node.
Nature of Specimen:
- Right breast mass, Referred block & slides (Tru-cut biopsy).
- Right axillary lymph node, Referred block & slide (U/S guided FNA, Cytology).

Gross Examination:
Two referred blocks and six referred slides were received as follow:
- Right breast mass: One referred block labelled by (200036-26), together with five referred slides including one (H&E) stained slide and four immunostained slides for (ER, PR, Her2 & SISH).
- Right axillary lymph node: One referred block and one referred (H&E) stained slide, labelled by (200013-26) (Cell block).

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; SISH.

Microscopic Examination:
- Right breast mass:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* IPT:
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Patient Name	: الأستاذه / امل عصام عيد الحميد	Request No.	: 13004805
Patient No.	: 27109040102667	Request Date	: 01/02/2026
Path No.	: SR-170/26	Verification Date	: 12/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Intermediate incomplete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH (Repeated in our lab): Amplified with small clusters.

- Right axillary lymph node:
Positive for malignant cells showing high nucleo/cytoplasmic ratio.

Diagnosis:
- Right breast mass, Referred block & slides (Tru-cut biopsy):
Invasive duct carcinoma, Grade 2.
IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Equivocal (score 2+).
- SISH (Repeated in our lab): Amplified with small clusters.

- Right axillary lymph node, Referred block & slide (Cell block of FNAC):
Positive for malignant cells.

** Comment (Ammended report issued to correct the amplification status):
- Initial analysis reported as non-amplified. Upon re-evaluation, focal areas of small clusters are noted, confirm amplification.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / امل عصام عبد الحميد	Request No.	: 13004805
Patient No.	: 27109040102667	Request Date	: 01/02/2026
Path No.	: SR-170/26	Verification Date	: 12/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ebrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / امل عصام عبد الحميد	Request No.	: 13004805
Patient No.	: 27109040102667	Request Date	: 01/02/2026
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Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name:	**AMAL ESSAM ABD ELHAMID AHME**	Gender:	**Female**
MRN:	**20226027422**	Ordered By:	**Mariam Ahmed Ibrahem Ahmed**
DOB:	**04/09/1971** Age: **54 Y 4 M 24 D**	Order ID:	**321529**

2D mammography

Procedure Date: 03/01/2026 11:17:00 AM

Report:

Report:
FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND
INDICATION:
- Right palpable breast mass .
TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound
REPORT:
- Right breast :
- Scattered fibroglandular densities (ACR B).
- 9 o'clock segmetal grouped plemorphic microcalcifications , extended of area (10.5x3.3 cm) . 4.5 cm away from nipple with underlying distortion
- This was confirmed by Complementary Ultrasound which showed 9 o'clock altered parenchyma coasistent with microcalcifications noted in mammography .\T\ spiculated hypoechoic mass (1.4x1 cm) , 7 cm away from the nipple , 0.3 cm from the skin , , another adjacent smaller mass closely related to the index mass , measures (0.4x0.2 cm)
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Indeterminate one axillary lymph node level 1 , measures (2.4x0.8 cm) with increased cortical thickness 0.4 cm .
- Left breast :
- Scattered fibroglandular densities (ACR B).
- Small circumscribed 1 o'clock dense mass noted to be small cyst by ultrasound
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.
OPINION:
- Right breast : 9 o'clock suspicious microcalcifications with suspicious masses , indeterminate lymph node (BIRADS 4C) , for histopathological correlation
- Left breast : no suspicious findings (BIRADS2).

Approved by: Shereen El Sharkawy,FEBR

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: AMAL ESSAM ABD ELHAMID AHMED
GENDER: Female
DATE : 03-Jan-2026 11:23 AM

PATIENT ID: 20226027422
AGE: 54 y
ORDER: Z321529,Z321530

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right palpable breast mass .

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- **Right breast :**
- Scattered fibroglandular densities (ACR B).
- 9 o'clock segmetal grouped plemorphic microcalcifications , extended of area (10.5x3.3 cm) . 4.5 cm away from nipple with underlying distortion
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- Small circumscribed 1 o'clock dense mass noted to be small cyst by ultrasound
- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Right breast : 9 o'clock suspicious microcalcifications with suspicious masses , indeterminate lymph node (BIRADS 4C) , for histopathological correlation
- Left breast : no suspicious findings (BIRADS2).

RECOMMENDATION:

Referral to clinic

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مستشفى بهية الشيخ زايد
٤ الحي الأول الشيخ زايد أمام كلية الهندسة
[[DOCTYPE:MAMMOGRAM]]

PATIENT NAME: AMAL ESSAM ABD ELHAMID AHMED **GENDER:** Female **DATE :** 28-Jan-2026 03:17 PM	**PATIENT ID:** 20226027422 **AGE:** 54 y **ORDER:** Z337891,Z337892

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC ASSESSMENT OF THE LIVER

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

Known case of right breast cancer

1-CHEST

- Right breast UOQ area of heterogeneous nodular parenchyma with internal calcifications.
- Enlarged right axillary level I & II lymph nodes, the largest seen at level I measuring 1.5 x 1 cm.
- Prominent left axillary level I and II lymph nodes.
- Prominent left internal mammary lymph node measuring 0.7 x0.4 cm.
- Bilateral basal subpleural reticulations intermingled with ground glass densities.
- Left upper lobe posterior segment focal minimal plural thickening with calcification
- No pulmonary masses, areas of consolidation or cavitary lesions.
- Right hilar region lymph node measuring about 23 x 18 mm.
- No evidence of pleural effusion or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles' dilatation..

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4 شارع علوية أول شارع الهرم بجوار داري , الجيزة
مستشفى بهية الشيخ زايد
١٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

CT Scan
[[DOCTYPE:CT_SCAN]]